



PO No :PO10003076399-945



Customer Name	: Mr.RAVINDERKUMAR	Collected Via	: TATA 1MG PUNE
Age/Gender	: 33Y 6M 17D /Male	Referred By	: Dr.
Lab Visit ID	: MGP940375	Collection Date	: 18/Jul/2026 08:48AM
Barcode ID/Order ID	: D31037016 / 17725890	Report Date	: 18/Jul/2026 02:20PM
Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Complete Blood Count				
Hemoglobin	14.2	g/dL	13.0-17.0	Spectrophotometry (Cyanide-free)
RBC	4.68	mili/cu.mm	4.5 - 5.5	Impedence
HCT	42.5	%	40 - 50	Calculated
MCV	90.9	fL	83 - 101	Calculated
MCH	30.4	pg	27 - 32	Calculated
MCHC	33.5	g/dL	31.5 - 34.5	Calculated
RDW-CV	14.4	%	11.5-14	Calculated
Total Leucocyte Count	6.64	10 ³ /μL	4 - 10	Impedance
Differential Leucocyte Count				
Neutrophils	51	%	40-80	DHSS/Microscopy
Lymphocytes	35	%	20-40	DHSS/Microscopy
Monocytes	7	%	2-10	DHSS/Microscopy
Eosinophils	6	%	1-6	DHSS/Microscopy
Basophils	1	%	0-2	Impedance / Microscopy
Absolute Leucocyte Count				
Absolute Neutrophil Count	3.39	10 ³ /μL	2 - 7	Calculated
Absolute Lymphocyte Count	2.32	10 ³ /μL	1-3	Calculated
Absolute Monocyte Count	0.46	10 ³ /μL	0.2 - 1	Calculated
Absolute Eosinophil Count	0.4	10 ³ /μL	0.02 - 0.5	Calculated
Absolute Basophil Count	0.07	10 ³ /μL	0.02-0.1	Calculated
Platelet Count	200	10 ³ /μL	150-410	Impedance/Microscopy
MPV	11.8	fL	6.5 - 12	Calculated
PDW	25.3	fL	9 - 17	Calculated

Comment:

As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.

DHSS : Double Hydrodynamic Sequential System Flowcytometry

Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

NABL certificate and scope



This test has been performed at

TATA 1MG PUNE

Teerth Business Center, Ground Floor, Unit
No 1, Sub-Divided Plot No EL-15, Bhosari
MIDC, Pune - 411026

Dr. PUNAM MOTIRAM RAUT
MBBS, MD (Pathology)
Consultant Pathologist
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HAEMATOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Erythrocyte Sedimentation Rate				
Erythrocyte Sedimentation Rate	7	mm/hr	0-10	Modified Westergren

Comment:

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis, acute rheumatic fever, Hodgkins disease, temporal arthritis, and systemic lupus erythematosus; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection, malignancy, pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyl dopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

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Barcode ID/Order ID	: D31037016 / 17725890	Report Date	: 18/Jul/2026 04:37PM
Sample Type	: Whole Blood-EDTA	Report Status	: Final Report

HAEMATOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Blood Group ABO & Rh Factor				
Blood Group (ABO typing)	O			Tube agglutination: Forward and reverse grouping
Rh Factor	Negative			

Comment:

Blood group is identified by antigens and antibodies present in the blood. Antigens are protein molecules found on the surface of red blood cells. Antibodies are found in plasma. To determine blood group, red cells are mixed with different antibody solutions to give A,B,O or AB. The test is performed by both forward as well as reverse grouping methods. The report is of sample received. It is presumed that the sample belongs to the patient.

Notes

- Reconfirm the Blood Group & Rh Typing along with cross matching before blood transfusion.
- Recent blood transfusion, if any, interferes with interpretation of blood grouping.
- The blood group and Rh antigen may change in the new born, hence please repeat the test after 6 months.
- Subgroups and Bombay blood group needs to be further verified

In case of any discrepancy related to this report, contact care@1mg.com

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Sample Type	: WHOLE BLOOD-EDTA	Report Status	: Final Report

HAEMATOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
HbA1c (Glycosylated Hemoglobin)				
Glycosylated Hemoglobin (HbA1c)	5.1	%	4 - 5.6	HPLC (NGSP certified)
Estimated average glucose (eAG)	99.67	mg/dL		Calculated

Comment:

Interpretation: HbA1c%

≤5.6	Normal
5.7-6.4	At Risk For Diabetes
≥6.5	Diabetes

Adapted from American Diabetes Association.

Comments:

A 3 to 6 monthly monitoring is recommended in diabetics. People with diabetes should get the test done more often if their blood sugar stays too high or if their healthcare provider makes any change in the treatment plan. HbA1c concentration represent the integrated values for blood glucose over the preceding 8-12 weeks and is not affected by daily glucose fluctuation, exercise & recent food intake.

Please note, Glycemic goal should be individualized based on duration of diabetes, age/life expectancy, comorbid conditions, known CVD or advanced microvascular complications, hypoglycemia unawareness, and individual patient considerations.

Factors that interfere with HbA1c Measurement: Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements.

Factors that affect interpretation of HbA1c Measurement: Any condition that shortens erythrocyte survival or decrease mean erythrocyte age (e. g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c.

Note: Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1c result does not correlate with the patient's blood glucose levels.

- HPLC - High performance liquid chromatography

NABL certificate and scope



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Barcode ID/Order ID	: D31037014 / 17725890	Report Date	: 18/Jul/2026 01:06PM
Sample Type	: Fluoride Plasma F	Report Status	: Final Report

BIOCHEMISTRY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
FBS (Fasting Blood Sugar)				
Glucose - Fasting	83	mg/dL	70-99	Hexokinase/G-6-PDH

Fasting Plasma Glucose (mg / dL)	2 hr plasma Glucose (mg / dL)	Diagnosis
99 or below	139 or below	Normal
100 to 125	140 to 199	Pre-Diabetes (IGT)
126 or above	200 or above	Diabetes

Reference : American Diabetes Association

Comment:

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes. IGT (2 hrs Post meal), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

Plasma Glucose Goals	For people with Diabetes
Before meal	70-130 mg/dL
2 Hours after meal	Less than 180 mg/dL
HbA1c	Less than 7 %

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Sample Type	: Serum	Report Status	: Final Report

BIOCHEMISTRY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Lipid profile				
Lipid Profile				
Cholesterol - Total	196	mg/dL	Low (desirable): < 200 Moderate (borderline) 200–239 High: >= 240	Enzymatic
Triglycerides	99	mg/dL	Normal: <150, Borderline: 150 - 199, High:200-499, Very High>=500	Glycerol Phosphate Oxidase
Cholesterol - HDL	48	mg/dL	High risk <40mg/dL Low risk >=40mg/dL	Accelerator Selective Detergent
Cholesterol - LDL	128	mg/dl	Desirable: <100 Above desirable: 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >=190	Calculated
Cholesterol- VLDL	20	mg/dl	<30	Calculated
Cholesterol : HDL Cholesterol	4.0	Ratio	Desirable : 3.5-4.5 High Risk : >5	Calculated
LDL : HDL Cholesterol	2.64	Ratio	Desirable : 2.5-3.0 High risk : >3.5	Calculated
Non HDL Cholesterol	148	mg/dL	Desirable:< 130, Above Desirable:130 - 159, Borderline High:160 - 189, High:190 - 219, Very High: >= 220	Calculated

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BIOCHEMISTRY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
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Comment:

- Lipid results show analytical and biological variation; repeat testing may be recommended before diagnosis or treatment decisions.
- Indians lie at high risk of developing early (a decade earlier than western populations) and more severe cardiovascular disease (ASCVD); higher mortality. Dyslipidemia (abnormal lipid profile) affects nearly 80% of population.
- Total cholesterol** is the sum of all cholesterol in the blood, including HDL, LDL, VLDL, and remnants.
- LDL Cholesterol (LDL-C)** is the major atherogenic lipoprotein associated with increased cardiovascular risk; generally calculated using the Friedewald equation. Direct LDL-C measurement is performed when triglycerides are ≥ 300 mg/dL or dysbetalipoproteinemia is suspected to ensure analytical accuracy.
- High-density lipoprotein (HDL)** or "good" cholesterol is anti-atherogenic (protective). Low HDL-C is a cardiovascular risk factor; seen in almost two-third of Indians. Values above 60 mg/dl are considered protective.
- Triglyceride (TG)** are a key driver of CVD. Indians are especially prone to atherogenic dyslipidemia—high TG, low HDL-C, and high LDL-C—closely linked to diabetes, metabolic syndrome, and insulin resistance; making TG management crucial.
- Non-HDL-Cholesterol (non-HDL-C)** Non-HDL-C measures all plaque-forming lipoproteins and is vital to monitor in high-TG patients (e.g., diabetics, obese) and those on statin therapy.
- Lipid Association of India (LAI-2020) recommends:-**
 - Screening of all Indians above the age of 20 years for CVD risk factors, esp. lipid profile.
 - Identification of Major Risk factors: Age (male ≥ 45 years, female ≥ 55 years); Family h/o heart disease at younger age (< 55 yrs in males, < 65 yrs in female or before menopause), current smoking/tobacco use, High blood pressure, Low HDL (males < 40 mg/dl and females < 50 mg/dl).
 - Fasting not mandatory; both fasting and non-fasting lipid profiles are useful for screening in Indian patients.
 - LAI identifies both LDL-C and non-HDL-C as risk factors and recommends LDL-C, non-HDL-C and Apo-B as targets of lipid-lowering therapy.
 - Lifestyle changes are the first-line approach for managing and preventing dyslipidemia. Treatment in low-risk individuals is initiated only after 3 months of unsuccessful lifestyle modification.
 - Testing for Apolipoprotein B(Apo-B), hsCRP, Lp(a) should be considered for patients in moderate risk group.

Treatment targets for lipid-lowering therapy for various ASCVD risk groups

Risk Group	Treatment targets		
	LDL-C, mg/dL (primary target)	Non-HDL-C, mg/dL (co-primary target)	Apo-B, mg/dL (secondary target)
Low-risk group	< 100	< 130	< 90
Moderate-risk group	< 100 (optional < 70)	< 130 (optional < 100)	< 90
High-risk group	< 70	< 100	< 80
Very high-risk group	< 50	< 80	< 65
Extreme-risk group- category A	< 50 (optional ≤ 30)	< 80 (optional ≤ 60)	< 65
Extreme-risk group- category B	≤ 30	≤ 60	< 50
Extreme-risk group- category C	10-15	40-45	-

Source: LAI (2024) Consensus Statement IV

- Per **NCEP Expert Panel (2011) guidelines**, universal dyslipidemia screening is advised at 9–11 years and repeated at 17–21 years. Screening before age 2 yrs is not recommended; from age 2 onward, selective screening is done for children with a family history of premature CVD or risk factors such as obesity, diabetes, or hypertension.

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BIOCHEMISTRY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
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Note: Biological Reference Interval as per National Cholesterol Education Program (NCEP) ATP III and LAI guidelines

LIVER FUNCTION TEST

Liver Function Test

Bilirubin-Total	0.53	mg/dL	0.3 - 1.2	Diazonium Salt
Bilirubin-Direct	0.23	mg/dL	0.0-0.5	Diazo Reaction
Bilirubin-Indirect	0.30	mg/dL	0.2-0.8	Calculated
Protein, Total	7.28	g/dL	6.4 - 8.3	Biuret, End Point
Albumin	4.88	g/dL	3.5-5.0	Bromocresol Green
Globulin	2.4	g/dL	2.1 - 3.9	Calculated
A/G Ratio	2.03	Ratio	0.8 - 2.1	Calculated
SGOT (Aspartate Aminotransferase)	35	U/L	11-34	NADH (Without P-5-P)
SGPT (Alanine Transaminase)	58	U/L	0-45	NADH (Without P-5-P)
SGOT/SGPT	0.61	Ratio		Calculated
Alkaline Phosphatase	114	U/L	50 - 116	Para-Nitrophenyl Phosphate
Gamma Glutamyltransferase (GGT)	37	U/L	12 -62	L-G-G-3-C-4-N Substrate

Comment:

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis

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BIOCHEMISTRY

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Test Name	Result	Unit	Bio. Ref. Interval	Method
(e.g., gallstones, tumors).				
• High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation, hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake, tissue damage, malabsorption, or increased renal excretion.				

Kidney Function Test

Kidney Function Test.

Blood Urea Nitrogen	12	mg/dL	6- 20	Urease
Urea	24.87	mg/dL	12.84-42.80	Calculated
Creatinine	0.80	mg/dl	0.6-1.2	Kinetic Alkaline Picrate
Uric Acid	6.4	mg/dL	3.7 - 7.7	Uricase
Sodium	139	mmol/L	136 - 145	Indirect ISE
Potassium	4.65	mmol/L	3.5-5.1	Indirect ISE
Chloride	106.7	mmol/L	98-107	Indirect ISE
BUN/Creatinine Ratio	14.5	Ratio	12:1 - 20:1	Calculated

Comment:

BUN is directly related to protein intake and nitrogen metabolism and inversely related to the rate of excretion of urea. Blood urea nitrogen (BUN) levels reflect the balance between the production and excretion of urea. Increased levels are seen in renal failure (acute or chronic), urinary tract obstruction, dehydration, shock, burns, CHF, GI bleeding, nephrotoxic drugs. Decreased levels are seen in hepatic failure, nephrotic syndrome, cachexia (low-protein and high-carbohydrate diets).

Urea is a non-proteinous nitrogen compound formed in the liver from ammonia as an end product of protein metabolism. Urea diffuses freely into extracellular and intracellular fluid and is ultimately excreted by the kidneys. Increased levels are found in acute renal failure, chronic glomerulonephritis, congestive heart failure, decreased renal perfusion, diabetes, excessive protein ingestion, gastrointestinal (GI) bleeding, hyperalimentation, hypovolemia, ketoacidosis, muscle wasting from starvation, neoplasms, pyelonephritis, shock, urinary tract obstruction, nephrotoxic drugs. Decreased levels are seen in inadequate dietary protein, low-protein/high-carbohydrate diet, malabsorption syndromes, pregnancy, severe liver disease, certain drugs.

Creatinine is catabolic product of creatinine phosphate, which is excreted by filtration through the glomerulus and by tubular secretion. Creatinine clearance is an acceptable clinical measure of glomerular filtration rate (GFR). Increased levels are seen in acute/chronic renal failure, urinary tract obstruction, hypothyroidism, nephrotoxic drugs, shock, dehydration, congestive heart failure, diabetes. Decreased levels are found in muscular dystrophy.

BUN/Creatinine ratio (normally 12:1-20:1) is decreased in acute tubular necrosis, advanced liver disease, low protein intake, and following hemodialysis. BUN/Creatinine ratio is increased in dehydration, GI bleeding, and increased catabolism.

Uric acid levels show diurnal variation. The level is usually higher in the morning and lower in the evening. Increased levels are seen in starvation, strenuous exercise, malnutrition, or lead poisoning, gout, renal disorders, increased breakdown of body cells in some cancers (including leukemia, lymphoma, and multiple myeloma) or cancer treatments, hemolytic anemia, sickle cell anemia, or heart failure, pre-eclampsia, liver disease (cirrhosis), obesity, psoriasis, hypothyroidism, low blood levels of parathyroid hormone (PTH), certain drugs, foods that are very high in purines - such as organ meats, red meats, some seafood and beer. Decreased levels are seen in liver disease, Wilson's disease, Syndrome of inappropriate antidiuretic hormone (SIADH),

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certain drugs.				



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BIOCHEMISTRY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
High Sensitive CRP				
High sensitivity CRP	0.98	mg/L	Healthy Individuals: <= 3.0 Low Risk: < 1.0 Average Risk: 1.0 to 3.0 High Risk: > 3.0	Turbidimetry

Comment:

Note:

1. Patients with persistently unexplained hs-CRP levels above 10 mg/L should be evaluated for other non-cardiovascular etiologies or sources of infection and inflammation.
2. For cardiovascular risk assessment, hs-CRP should ideally be measured twice, two weeks apart, and the average value is used (accounts for within-subject variability), in metabolically stable patient.

Interpretation:

- High-sensitivity C-reactive protein (hs-CRP) serves as a biomarker for cardiovascular risk assessment, typically evaluated in conjunction with the lipid profile.
- The American Heart Association and US Centers for Disease Control and Prevention have defined risk groups as follows: * <1.0 Low Risk; 1.0 - 3.0 - Average Risk; >3.0 High Risk
- Both hs-CRP and CRP measure the same protein, but hs-CRP is more sensitive and used for cardiovascular risk assessment. For accurate interpretation, testing should be done when the patient is in a stable, healthy state.
- Recent illness, infection, tissue injury, or other sources of inflammation may elevate hs-CRP and lead to an overestimation of cardiovascular risk; It should not be used in individuals with chronic inflammatory conditions (e.g., arthritis), as results may be misleading.
- Women on hormone replacement therapy have been shown to have elevated hs-CRP levels.
- Anti-inflammatory drugs (e.g., aspirin, ibuprofen, naproxen) and statins can lower CRP levels.

*Source: Pearson et al; Markers of inflammation and cardiovascular disease; Circulation, 107 (3) (2003)



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Age/Gender	: 33Y 6M 17D /Male	Referred By	: Dr.
Lab Visit ID	: MGP940375	Collection Date	: 18/Jul/2026 08:48AM
Barcode ID/Order ID	: D31037013 / 17725890	Report Date	: 18/Jul/2026 02:23PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin D (25-OH)				
Vitamin D (25-OH)	16.4	ng/mL	Deficiency:< 20, Insufficiency:20-29, Sufficiency:30 - 100, Toxicity possible:> 100	CMIA

Comment:

- Vitamin D is a fat-soluble steroid prohormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis.
- Two forms of vitamin D are biologically relevant - vitamin D3 (Cholecalciferol) and vitamin D2 (Ergocalciferol).
- Both vitamins D3 and D2 can be absorbed from food but only an estimated 10-20perc. of vitamin D is supplied through nutritional intake.
- Vitamin D is converted to the active hormone 1,25-(OH)2-vitamin D (Calcitriol) through two hydroxylation reactions. The first hydroxylation converts vitamin D into 25-OH vitamin D and occurs in the liver. The second hydroxylation converts 25-OH vitamin D into the biologically active 1,25-(OH)2-vitamin D and occurs in the kidneys as well as in many other cells of the body.
- Most cells express the vitamin D receptor and about 3perc. of the human genome is directly or indirectly regulated by the vitamin D endocrine system.
- The major storage form of vitamin D is 25-OH vitamin D and is present in the blood at up to 1,000 fold higher concentration compared to the active 1,25-(OH)2-vitamin D. 25-OH vitamin D has a half-life of 2-3 weeks vs. 4 hours for 1,25-(OH)2-vitamin D. Therefore, 25-OH vitamin D is the analyte of choice for determination of the vitamin D status.
- Risk factors for vitamin D deficiency include low sun exposure, inadequate intake, decreased absorption, abnormal metabolism, vitamin D resistance and liver or kidney diseases.
- Vitamin D deficiency is a cause of secondary hyperparathyroidism and diseases resulting in impaired bone metabolism (like rickets, osteomalacia).
- Recently, many chronic diseases such as cancer, high blood pressure, osteoporosis and several autoimmune diseases have been linked to vitamin D deficiency.
- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D

Utility Quantitative determination of 25-hydroxyvitamin D (25-OH vitamin D).

* **CMIA**-Chemiluminescent Microparticle Immunoassay / **CLIA**-Chemiluminescent immunoassay.



This test has been performed at
TATA 1MG PUNE
 Teerth Business Center, Ground Floor, Unit
 No 1, Sub-Divided Plot No EL-15, Bhosari
 MIDC, Pune - 411026

Dr. Sangeeta Parmar
 (M.D., Pathologist)
 Reg. No. 61255





PO No :PO10003076399-945



Customer Name	: Mr.RAVINDERKUMAR	Collected Via	: TATA 1MG PUNE
Age/Gender	: 33Y 6M 17D /Male	Referred By	: Dr.
Lab Visit ID	: MGP940375	Collection Date	: 18/Jul/2026 08:48AM
Barcode ID/Order ID	: D31037013 / 17725890	Report Date	: 18/Jul/2026 02:23PM
Sample Type	: Serum	Report Status	: Final Report

IMMUNOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Vitamin B12	190.3	pg/mL	187-833	CMIA

Comment:

- **Vitamin B12** along with **folate** is essential for DNA synthesis and myelin formation.
- **Decreased levels** are seen in anaemia, term pregnancy, vegetarian diet, intrinsic factor deficiency, partial gastrectomy/ileal damage, celiac disease, oral contraceptive use, parasitic infestation, pancreatic deficiency, treated epilepsy, smoking, hemodialysis and advanced age.
- **Increased levels** are seen in renal failure, hepatocellular disorders, myeloproliferative disorders and at times with excess supplementation of vitamins pills.

* **CMIA**-Chemiluminescent Microparticle Immunoassay / **CLIA**-Chemiluminescent immunoassay.NABL certificate
and scope

This test has been performed at

TATA 1MG PUNE

Teerth Business Center, Ground Floor, Unit
No 1, Sub-Divided Plot No EL-15, Bhosari
MIDC, Pune - 411026Dr. Sangeeta Parmar
(M.D., Pathologist)
Reg. No. 61255Scan for
digital copy



PO No :PO10003076399-945



Customer Name	: Mr.RAVINDERKUMAR	Collected Via	: TATA 1MG PUNE
Age/Gender	: 33Y 6M 17D /Male	Referred By	: Dr.
Lab Visit ID	: MGP940375	Collection Date	: 18/Jul/2026 08:48AM
Barcode ID/Order ID	: D31037015 / 17725890	Report Date	: 18/Jul/2026 03:24PM
Sample Type	: Urine	Report Status	: Final Report

CLINICAL PATHOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
Urine Routine & Microscopy				
Physical & Chemical Examination				
Colour	Pale Yellow		Pale Yellow	Visual
Appearance	Clear		Clear	Visual
Specific gravity	1.015		1.003 - 1.035	pKa change
pH	5.5		4.6-8.0	Double Indicator
Glucose	Negative		Negative	GOD-POD
Protein	Negative		Negative	Protein- error principle
Ketones	Negative		Negative	Nitroprusside
Blood	Negative		Negative	Peroxidase
Bilirubin	Negative		Negative	Diazonium
Urobilinogen	Normal		Normal	Azo Dye
Leucocyte Esterase	Negative		Negative	Pyrrole
Nitrite	Negative		Negative	Sulphanilamide Diazo
Microscopic Examination				
Pus cells	1-2	/hpf	0-5	Microscopy
Red Blood Cells	Nil	/hpf	0-2	Microscopy
Epithelial cells	1-2	/hpf	Few	Microscopy
Casts	Nil	/hpf	Nil	Microscopy
Crystals	Nil		Nil	Microscopy
Yeast	Nil		Nil	Microscopy
Bacteria	Nil		Nil	Microscopy

Comment:

•Note: Pre-test condition to be observed while submitting the sample-first void, mid stream urine, collected in a clean, dry, sterile container is recommended for routine urine analysis, avoid contamination with any discharge from vaginal, urethra, perineum, Avoid prolonged transit time & undue exposure to sunlight.
•During interpretation, points to be considered are Negative nitrite test does not exclude the urinary tract infections. Trace proteinuria can be seen with many physiological conditions like prolonged recumbency, exercise, high protein diet. False positive reactions for bile pigments, proteins, glucose and nitrites can be caused by peroxidase like activity by disinfectants, therapeutic dyes, ascorbic acid and certain drugs. • Urine microscopy is done in centrifuged urine specimens

NABL certificate and scope



This test has been performed at
TATA 1MG PUNE
Teerth Business Center, Ground Floor, Unit
No 1, Sub-Divided Plot No EL-15, Bhosari
MIDC, Pune - 411026

P. Raute

Dr. PUNAM MOTIRAM RAUT
MBBS, MD (Pathology)
Consultant Pathologist
Reg No : 2018041316

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PO No :PO10003076399-945



Customer Name	: Mr.RAVINDERKUMAR	Collected Via	: TATA 1MG PUNE
Age/Gender	: 33Y 6M 17D /Male	Referred By	: Dr.
Lab Visit ID	: MGP940375	Collection Date	: 18/Jul/2026 08:48AM
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Sample Type	: Urine	Report Status	: Final Report

CLINICAL PATHOLOGY

EK26-IT-01HC

Test Name	Result	Unit	Bio. Ref. Interval	Method
*** End Of Report ***				

Disclaimer:

1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
3. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

TATA 1mg Labs

NABL certificate
and scope

This test has been performed at
TATA 1MG PUNE
Teerth Business Center, Ground Floor, Unit
No 1, Sub-Divided Plot No EL-15, Bhosari
MIDC, Pune - 411026

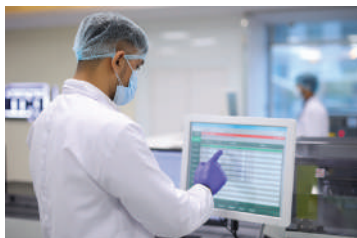
Dr. PUNAM MOTIRAM RAUT
MBBS,MD (Pathology)
Consultant Pathologist
Reg No : 2018041316

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Pharmacist/Sig.

SHREE NEW KOHINDOR CHEMTEST

Bill No: 16093

SHOP NO. 3 GR FLR AISHWARYA CORNER KATEPURAM CHOWK

Date: 13/08/2026

PIMPLE GRAY PUNE 411061 MOB-7219753341

Cash Memo

NAME :RAVINDER KUMAR / PIMPLE GRAY
DOCTOR:PRATIKTA CHAUDHARI / PUNE

Qty	Product	CompBatch	Exp	Amount
20XCAP	MEGANELFON OD PILLS	OTS SPD260639	03/28	358.00
10XCAP	MEGANELFON OD PILLS	OTS SPD260639	03/28	179.00

Gross Amt: 537.00

Net Amt: 537.00

E & O E. Subject to PCMC Jurisdiction

SHREE NEW KOHINDOR CHEMIST

MH. 20-558223 21-558224

Mr. Ravinder Kumar

Age: 33 y/o M

Height :	Weight :	BMI :	Waist Circum :
Temp :	Pulse :	Resp :	B.P :

General Examination / Allergies
History

Clinical Diagnosis & Management Plan

Rx

Cap. Upose D3 60 K once
a week after breakfast x 8 weeks
+ 4 months

Cap. Meganeuron OD +

after lunch x 1 month

Vaccination History

- ☐ Influenza Vaccine
- ☐ Pneumococcal Conjugate Vaccine
- ☐ Shingles Vaccine
- ☐ Tdap (Tetanus, Diphtheria, Pertussis) Vaccine
- ☐ HPV Vaccine
- ☐ Others

Follow up date:

Apollo Clinic - Aundh
Dr. PRAJAKTA CHAUDHARI
MBBS, MD (Physiology)
Family Physician and Wellness Consultant
Reg. No. 2004031662
Doctor Signature

Book Your Appointment

Whatsapp Number : 7066028648
For appointments call : 1860 500 7788
Website : www.apolloclinic.com

Address: Aundh

Centriole building above star bucks, Plot #90,
Survey #129, 130/1+2, ITI Road, Aundh - 411007
Phone: (020) 2588 7961 - 4



PUNE - AUNDH NAGRAS RD Branch
SHOP NO. 21-A, WESTEND RIVER VIEW, NAGRAS ROAD, AUNDH, PUNE - 411007
RTGS/NEFT IFS CODE : ICIC0001878

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